The Hillingdon Hospitals
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Perineal Care, Tears and Episiotomy Repair

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4.0	Gillian Pearce, Maternity Governance Midwife	Full review following RCOG guidance
5.0	Gillian Pearce, Maternity Governance Midwife	Amendment to proforma following a WHO audit
6.0	Giulia Ciccolini	Complete revision of guideline

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1 Aim/Purpose

This guideline applies to ALL women following vaginal birth, including assessment and repair of 1st/2nd/3rd/4th degree tears and an episiotomy.

2 Introduction

The aim of this guideline is to provide information on best practice and management of perineal tear at the time of delivery and in the postnatal period. Morbidity associated with perineal injury relating to childbirth constitutes a major health problem worldwide in the UK 350,000 women require suturing annually and 20% have long term problems (Kettle 2005). Correct management and repair is essential and an important aspect of maternal recovery.

The current rate of third and fourth degree tears or obstetric anal sphincter injuries (OASIS) in singleton, term, cephalic, vaginal first births in England has tripled from 1.8% to 5.9% since the last report (Royal College of Obstetricians and Gynaecologists (RCOG) 2015).

3 The Guidance

3.1 Definition and Diagnosis of Perineal Tears:

Perineal tears are classified according to the tissues involved (RCOG Green-top Guideline No. 29). The classification is based on a continuum e.g. there is no classification which recognises an isolated internal anal sphincter tear in the presence of an intact external anal sphincter. If there is any doubt about the degree of anal sphincter injury, it is advisable to classify it to the higher degree rather than the lower degree (RCOG Green-top Guideline No. 29).

1st degree tear:	Injury to perineum involving skin only and /or fourchette, vaginal mucous membrane. There is no involvement of the underlying fascia or muscle.
2nd degree tear:	Injury to perineum involving the superficial perineal muscles (Bulbocavernosus and Transverse Perineal) but does not involve the anal sphincter.
3rd degree tear:	Injury to perineum involving the anal sphincter complex, including partial or complete involvement of the external anal sphincter (EAS) and/or internal anal sphincter (IAS). 3A less than 50% of EAS torn 3B more than 50% of EAS torn 3C IAS and EAS torn
4th degree tear:	Injury to perineum involving the anal sphincter complex (EAS and IAS) and rectal mucosa.
Rectal buttonhole tear:	Isolated tear in the rectal mucosa with or without third degree tear If the tear involves the rectal mucosa with an intact anal sphincter complex, it is by definition not a fourth-degree tear. This must be documented as a rectal buttonhole tear. If not recognised and/or appropriately repaired, this type of tear may lead to a rectovaginal fistula.

3.2 Episiotomy

An episiotomy is defined as a deliberate incision through the skin and muscles of the perineum in order to enlarge the vaginal orifice, and involves the same structures as a second-degree tear. A **mediolateral** episiotomy is favoured as this is less likely to increase the risk of anal sphincter damage, (Wooley 1995). This involves an incision starting from the midline at the posterior fourchette at a 60° angle when the head is crowning. There is no evidence to support the routine use of episiotomy for normal vaginal births. Episiotomies should only be performed if there is a clinical indication for the procedure. These include instrumental delivery, foetal distress, a rigid inelastic perineum or to facilitate delivery e.g shoulder dystocia, breech delivery.

The patient should be informed and verbal consent obtained. It must be done only when the presenting part is distending the perineal tissue to avoid bleeding and with the woman adequately anaesthetised.

3.3 Protection of the perineum

The individual practitioner can opt to choose between ‘hands poised’ (with hands off the perineum and baby's head but in readiness to guard the perineum) and ‘hands on’ (guarding the perineum and flexing the baby's head) technique as intrapartum interventions to reduce perineal trauma as recommended by NICE 2014.

Warm compress

There is the option to consider the application of warm compresses during the second stage of labour as it may reduce OASIS, (Cochrane review, 2011). The intervention involves holding the compress on the perineum continuously during and between contractions. Attention to the temperature of the warm compress is important to avoid maternal injury, particularly in those with an epidural.

Antenatal perineal massage

Perineal massage from 35 weeks of pregnancy has been suggested as a possible way of enabling perineal tissue to expand more easily during birth. There is evidence (Cochrane review, 2013) showing that perineal massage undertaken by the woman or her partner was associated with an overall reduction in the incidence of tears requiring suturing. Women practising perineal massage were less likely to have an episiotomy. These findings were significant for women without previous vaginal birth only. No differences were seen in the incidence of first- or second-degree perineal tears or third-/fourth-degree perineal trauma.

OASI care bundle

The OASI care bundle is a set of four interventions in order to reduce the risk of OASIs when implemented together. The elements of the care bundle were selected based on recommendations from existing UK guidelines. The elements of the OASI bundle included:

1. Antenatally, informing woman about OASI and identifying what can be done to minimise her risk
2. If clinically indicated and with the woman’s consent, performing an episiotomy at an angle of 60 degrees from the midline at crowning.

3. Using the hands to support the perineum at the time of birth (known as manual perineal protection, or MPP)
4. A thorough combined rectal and vaginal examination after birth to detect tears, not all of which will be immediately obvious.

All elements of the care bundle are underpinned by good communication with the woman before, during and after birth. It is important that all four elements of the care bundle are used consistently provided that the woman has had the opportunity to discuss the care bundle with the provider during antenatal care and has given her consent to its use during her birth.

Through implementation of the care bundle, it was shown that the risk of an OASI occurring was reduced by 20%, with no corresponding rise in caesarean births or episiotomy rates.

At Hillingdon Hospital, we are in the process of developing and piloting an intervention package which includes a skills development module for clinicians, materials to raise awareness of the OASI care bundle among clinicians and women and also face-to-face specialist hands on training for clinicians on how to implement the OASI care bundle.

3.4 Who can perform the repair

1st & 2nd Degree

Practitioners who have undergone training in perineal suturing are more likely to provide a consistent high standard of perineal repair. Therefore, only those obstetric specialty doctors and midwives who have completed appropriate training can perform suturing, as outlined in the Training Needs Analysis or are deemed competent by other methods. Clinicians who are undergoing training must only operate with one-to-one supervision. Seek advice from a more experienced midwife or obstetrician if there is uncertainty about the nature or extent of tear (NICE CG190 Intrapartum care for healthy women and babies).

3rd & 4th Degree

Obstetric specialty doctors will have their training reviewed when they start in post by the Clinical Tutor or deputy. An obstetric speciality doctor with appropriate training and experience can suture 3rd and 4th degree tears. This will be as a minimum ST3 and above or trust doctor. However, ST2 with ST7 or consultant supervision may be appropriate. All 4th degree tear repairs must be supervised by a consultant. Formal training in anal sphincter repair techniques should be an essential component of obstetric training (RCOG Green-top Guideline No. 29).

Rectal buttonhole tears

An obstetric speciality doctor with appropriate training and experience can suture a rectal buttonhole tear. However, a consultant should **always** be present to supervise. Colorectal involvement should always be sought for high rectal buttonhole tears (> 7 cm from the anal verge or if there is faecal soiling). A covering colostomy is rarely performed but may be considered in these scenarios. The risks and benefits of colostomy should be discussed with the patient (Roper 2020).

3.5 Systematic assessment of the perineum & lower vagina

INITIAL ASSESSMENT: All women giving birth vaginally must be assessed for vaginal and perineal tear. This must be carried out as soon as possible after birth ensuring verbal consent has been obtained ideally the assessment will not interfere with mother-infant bonding unless the woman is bleeding and requires urgent attention. The initial assessment must be performed by trained clinician, either a doctor or midwife. Documentation must be in the maternal notes.

Healthcare Professionals must:

- Explain the procedure to the woman and why it is required
- Gain verbal consent
- Document in the maternal notes (use "1st & 2nd Degree Tears/Episiotomy" or "3rd & 4th Degree Tears" proforma only if suturing is required)
- Offer analgesia when required (e.g. Entonox). Test effectiveness prior to commencing procedure.
- Ensure good lighting to assess extent of tear and bleeding.
- Position the woman appropriately for repair, lithotomy can be considered if appropriate.
- All women having a vaginal delivery are at risk of sustaining OASIS or isolated rectal buttonhole tears.

They should therefore be examined systematically, including a combined vaginal and digital rectal examination, to assess the severity of damage, particularly prior to suturing (RCOG Green-top Guideline No. 29). Perform a rectal examination to assess whether there has been any damage to the external or internal anal sphincter, if there is any suspicion that the perineal muscles are damaged or there is a rectal buttonhole tear prior to performing the vaginal repair**.**

NB Lithotomy position must only be maintained for as long as necessary for the systematic assessment and repair, the length of time must be documented in the maternal notes.

FURTHER ASSESSMENT:
If a 3rd or 4th degree tear is suspected, or if there is uncertainty as to the nature or extent of tear sustained, referral will be made to an appropriately trained obstetric speciality doctor ST3 or above.

In the home setting or midwifery led unit, a woman must be transferred to the Labour Ward if a 3rd or 4th degree tear is suspected so that a thorough assessment can be made.

3.6 When Non-Suturing may be applicable

If there is a 1st degree tear, it is dependent on haemostasis, cosmetic appearance and maternal choice, whether non-suturing may be applicable or not. First degree tears should be sutured unless the skin edges are well opposed and bleeding is minimal. Also there may be a maternal request for non-suturing and this is after an explanation for the reasons to repair the tear has been given to the woman and she subsequently chooses not to have a repair of the tear (1st, 2nd degree tear). The discussion regarding possible adverse outcomes and morbidity

should be documented in her notes. Labial lacerations should be sutured if the trauma is bilateral or bleeding as fusion may occur.

3.7 Methods and materials used in repair of 1st and 2nd degree tears or episiotomy

All 2nd degree perineal tears should be sutured to achieve haemostasis, to achieve anatomical alignment, to promote primary intention healing and to reduce infection. Suturing will take place soon after birth preferably within 1-2 hours of delivery after obtaining and documenting **informed verbal consent**. Suturing takes priority in the event of active bleeding occurring from the perineum -assess estimated blood loss and be aware of continuing bleeding. Maternal observations and action is required if bleeding is excessive.

Analgesia for 1st and 2nd degree tears or episiotomy:
See Midwifery Exemptions policy for the dosage of lidocaine. 1% lidocaine is to be injected into the area around the tear to ensure adequate analgesia for suturing. If an epidural is in situ, a top up can be given and Entonox can be used if required.

Repair in theatre with regional anaesthesia may be appropriate for 2nd degree tears that are complicated or require better lighting.

A swab count must be performed pre and post suturing and checked with a second person. Both counts must be documented on the proforma and signed by both staff members. See **'**Procedure for counting swabs, needles & instruments in Maternity - excluding theatres' guideline.

<u>1st degree</u>
Consider suturing skin with subcuticular braided polyglactin (Vicryl rapide 2.0) or any other appropriate suture material.

<u>2nd degree and Episiotomies</u>
Suture vaginal wall and muscle for **ALL** 2nd degree tears, suture skin **unless well opposed, to reduce long term morbidity**.

Repair Technique for 1st and 2nd degree tears or episiotomy:

- Good lighting should be in place before suturing is commenced
- Open the outer layer of the suture pack and the drapes. Put tap water in the bowl. Put on goggles. Wash hands as per the Trust Hand washing policy.
- Use an aseptic technique i.e. sterile gloves, open the remainder of the pack and proceed.

Equipment must be checked and swabs and needles counted before the procedure.

Identify the apex, inserting the first stitch above to secure any bleeding points. If you have difficulty seeing the apex, use a Sims speculum to lift the vaginal wall and call for assistance.

- If there is more than one tear, each apex should be individually identified and secured. It is important that the perineal body is reconstituted if disrupted.
- Undertake perineal repair using a continuous non-locking suturing technique for the vaginal wall and muscle layer (NICE CG190 Intrapartum care for healthy women and babies). Based on meta-analysis, this suturing technique is associated with less short-

term pain, need for analgesia and suture removal. However, compared with interrupted methods, there are no significant differences in the need for resuturing or long-term pain. (Kettle et al. 2012 Cochrane Database of Systematic Reviews).

- Suture the vaginal wall by putting the 1st stitch just above the apex and then suture continuously with a non-locking technique with an absorbable synthetic suture material (NICE CG190 Intrapartum care for healthy women and babies).
- Suture the perineal muscle using a continuous technique to re-approximate the vagina and perineal muscles. Ligate bleeding vessels with a figure of eight stitch. The dead space must be closed ensuring that haemostasis has been obtained.
- Where the skin does require suturing, a continuous subcuticular technique with 2.0 vicryl rapide or any other appropriate suture material is recommended. Studies have shown that subcuticular suturing is associated with less pain and requirement for analgesia post-partum than interrupted sutures to the skin. Also, the use of interrupted sutures may act as a nidus for bacterial growth and delay wound healing (Okeahialam 2022).
- Suture from the apex on the perineal body to the fourchette, good anatomical alignment of the wound must be achieved, and consideration given to the cosmetic results.
- Most perineal tears and episiotomies are effectively repaired using Vicryl Rapide 2/0 or 3/0 (or equivalent). Kettle et al. compared Vicryl (polyglactin) with Vicryl Rapide, reporting the need for less suture removal with the more rapidly absorbed Vicryl Rapide.
- **Check that all the swabs and needles are correct and document**
- **Examine the vagina and rectum** digitally to check there are no sutures in the rectal mucosa or any gaps in the vaginal mucosa, particularly at the apex. Vaginal and rectal examination are also important to exclude haematoma and ensure good alignment of tissue.
- Consider an indwelling catheter for 24 hours to prevent urinary retention in women with complex tears, for example if close to the urethral orifice. Document plan for removal of the catheter on the proforma **(Appendix 1)**.
- The woman must remain comfortable following the procedure. Offer rectal non-steroidal anti-inflammatory drugs routinely, provided they are not contraindicated e.g. asthma, allergy.

Equipment for Suturing & Suture Material:

- Needle holder
- Tissue forceps
- Stitch scissors
- Spencer Wells forceps +/- Sims speculum
- 2.0 Vicryl rapide (associated with less pain postnatally) or any other appropriate suture material.
- 20ml syringe
- Green needle
- Movable light source

3.8 Post Operative Care for 1st and 2nd degree tears or episiotomy

- Good anatomical alignment of the wound should be achieved with no excess bleeding and consideration given to the cosmetic results.

- At the completion of the repair a vaginal examination should be performed to ensure that the vaginal introitus admits at least two fingers.
- A rectal examination must be done to exclude any sutures in the anorectal mucosa. If sutures are identified (except in a 4th degree tear repair), the stitches should be removed, and the wound should be resutured with the midwife in charge/consultant and the woman should be informed.
- **Documentation of consent** and the repair process must be done on the proforma Vaginal and rectal examinations as well as correct suture and swab counts must be documented. (Appendix 1).
- All women who have received regional anaesthesia (epidural and/or spinal) or who have significant perineal trauma should have a urinary catheter remain in situ for AT LEAST 12 HOURS following delivery.
- PR Diclofenac Sodium 100mgs PR should be offered routinely after perineal tears unless this drug is not contraindicated. Analgesia must be prescribed appropriate to the woman's needs.
- An information leaflet (appendix 4) is given at discharge and documented on the postnatal discharge checklist. Women should be informed about the extent and repair of their perineal tear. They should be given advice and written information regarding perineal care and what to expect in the days and weeks following with respect to analgesia, diet, hygiene, micturition, defaecation and pelvic floor exercises.

The woman must be encouraged to seek advice if she has problems. This may be from the community midwife up to day 10, through triage up to day 28 or from the GP. Health care professionals can refer client to Perinatal Pelvic Health Service as per inclusion criteria using the appropriate referral form (appendix 5).

In the postnatal period, all women's perineum should be routinely assessed by the relevant clinician as part of the postnatal check. Assessment of the healing process should be also made by asking if there is pain, discomfort, stinging or offensive odour. To facilitate the documentation of perineal inspection, please find the sticker on the Appendix 6.

When the assessment takes place in community, If there is infection, inadequate repair, wound breakdown or poor healing, the woman should be referred to Triage, her GP or Accident and Emergency for urgent review. If perineal wound infection or any perineal problem is suspected, the woman must also be referred to the Perinatal Pelvic Heath Service team after the urgent assessment (please see paragraph 3.8.1 and 3.12 for further details).

3.8.1 Perinatal Pelvic Health Service (PPHS)

Perinatal Pelvic Health Services (PPHS) delivers treatments (appendix 7) locally with the aim to improve the prevention, identification and treatment of 'mild to moderate' pelvic floor dysfunction following birth, and ultimately reduce the number of women living with pelvic floor dysfunction postnatally and in later life.

The clinicians who include a urogynaecologist and obstetrician consultant, a specialist midwife and a specialist physiotherapist will provide pelvic health care with a standardised process to:

- Pregnant people with conditions affecting their pelvic health

• People who have given birth within the last 12 months who have conditions affecting their pelvic health.

Referral Pathway:

There is a standardised proforma (Appendix 5) for referral to PPHS for pregnant and postnatal women.

Referral forms can also be found on the Maternity share drive and should be sent via email to thh.perinatalpelvichealthservice@nhs.net .

Each referral will be vetted to ensure it meets the criteria. Each referral will receive a grade according to the urgency of care required. Time of referral to review will be within 4 weeks.

After vetting, an appointment will be offered to see the PPHS Multi-Disciplinary Team Clinic.

Women will be informed about the service at their booking appointment. During the antenatal booking appointment, the booking midwife will ask questions about pelvic health and a page of the maternity handhold notes and Euroking will be completed for each woman. These questions aim to address any signs or symptoms of pelvic health dysfunction that require a prompt referral to the PPHS.

Referral Source:

- Antenatal and postnatal clinic/community staff.
- Maternity day assessment unit/triage staff
- Labour ward and midwifery led unit staff
- Antenatal/Postnatal ward staff
- General practitioners
- Health visitors

There will be a clinic at least once a week, every Thursday running between 9.30am to 12.30 pm in the Cumberlege Unit.

An MDT meeting will be held once a month to discuss complex cases and to consider referral to the specialist OASI clinic, FGM, psychosexual, urogynaecology or colorectal services

The clinic will have a nominated referral pathway into the psychological and birth trauma services for women requiring further input.

In addition, there will be biannual auditing reflecting local incidences of OASI, perineal breakdown and outcomes, sexual dysfunction, incontinence treatment and recovery.

3.9 Management of 3rd and 4th degree tears

- An obstetric speciality doctor with appropriate training and experience can suture 3rd and 4th degree tears. This will be as a minimum ST3 and above, including trust registrars. However, ST2 with ST6/ ST7 or consultant supervision may be appropriate. Consultant must be present during repair of 4th degree tear.

- A specially prepared pack is available containing the necessary surgical instruments.
- The repair should be performed under regional block i.e. epidural or spinal. Attention should be paid to blood loss and appropriate measures taken as necessary. Delays in transfer to theatre will increase the chances of significant blood loss and need for transfusion."
- 3rd and 4th degree perineal tears should be repaired in theatre with good light. Repair of OASIS in the in the delivery room may be performed in certain circumstances after discussion with a senior obstetrician (RCOG Green-top Guideline No. 29).
- Emotional support of the mother is paramount; the woman must be supported whilst the repair is being carried out.
- The proforma must be completed including obtaining **informed written consent** with the names and signatures of both doctors if applicable **(Appendix 2)**.

A swab count must be performed pre and post suturing and checked with a second person. Both counts must be documented on the proforma and signed by both staff members.

Repair Technique for 3rd and 4th degree tears:

- Interrupted or continuous 3.0 Vicryl rapide (polyglactin) to the rectal mucosa with knots tied inside the anal canal
- End-to-end technique using 3.0 PDS or 2.0 Vicryl to IAS muscles
- EAS muscles can be repaired with either overlap or end-to-end technique using 3.0 PDS (polydioxanone) or 2.0 Vicryl
    - An overlap repair can only be used if the full thickness and length of the EAS is torn. Therefore, 3a tears can only be repaired using an end-to-end technique.
- Continuous non-locking 2.0 Vicryl rapide to the vaginal epithelium
- Continuous non-locking 2.0 Vicryl rapide to perineal muscles, take care to avoid creating a deficient perineum which increases the risk of further trauma in a subsequent delivery
- Ensure that the knots and suture ends of PDS sutures are completely buried with overlying tissue to avoid suture migration
- Subcuticular 2.0 vicryl rapide to the perineal skin

(Alternative appropriate perineal sutures can be used)>

NB. Figure of eight sutures should be avoided during the repair of the anal sphincter muscels because they are haemostatic in nature and may cause tissue ischaemia.

Repair Technique for isolated rectal buttonhole tears (Roper 2020)

- A three-layered approach should be used
- The rectal mucosa should be repaired using a transvaginal approach, with a continuous non-locking 3–0 Vicryl, with knots on the vaginal side of the rectal tear. Perform a rectal examination to confirm that good apposition of the mucosa is obtained
- Use an interrupted mattress technique using a 2–0 or 3–0 PDS sutures to the rectovaginal fistula.
- Continuous non-locking 2–0 Vicryl sutures to the vaginal skin
- A digital vaginal and rectal examinations should be performed to ensure complete closure of the tear

Post Operative Care for 3rd and 4th degree tears:
Vaginal and rectal examinations as well as correct suture and swab counts must be documented on the proforma **(Appendix 2)**. Severe perineal discomfort particularly following instrumental delivery is a known cause of urinary retention and following regional anaesthesia it can take up to 12 hours before bladder sensation returns. Therefore a Foley's catheter should be left in for up to 24 hours. The following must be prescribed:

- Oral Lactulose 10ml bd for 10 days
- Co-amoxiclav 1.2g IV in theatre and follow "Antibiotic Guideline for Obstetrics and Gynaecology")
- Co-amoxiclav orally for 7 days (see "Antibiotic Guidelines for Obstetrics and Gynaecology" guideline)
- Oral analgesia (paracetamol 1g qds and diclofenac MR 75mg bd for 3 days, avoid codeine-based preparations, avoid suppositories and enemas if opioids are prescribed.
- Give the woman the patient information leaflet (Appendix 3) and offer appropriate information leaflet on pelvic floor exercise and document.
- A datix must be completed in line with the Maternity Service Risk Management Strategy

Process for offering postnatal appointment for 3rd and 4th degree tears:
The woman must be encouraged to seek advice if she has problems. This may be from the community midwife up to day 10, through triage up to day 28 or from the GP. An **information leaflet (appendix 3)** is given on discharge. This is documented on the postnatal discharge checklist.

- The woman will have a 6-11 week follow-up appointment with the consultant and specialist pelvic health physiotherapist on the same day.
- If there are symptoms of bowel dysfunction at the follow-up appointment, the consultant will arrange an onward referral to St Marks Northwick Park Hospital for an endoanal ultrasound scan and anorectal physiology tests which will lead the decision on next mode of delivery.
- At Hillingdon Hospital, asymptomatic women who have sustained a 3a, 3b or 3c tear have a mode of delivery recommendation in a subsequent birth made with the perinatal midwife/OASI consultant. All symptomatic women and all 4th degree tears must be referred to St Marks for an endoanal ultrasound scan and anorectal physiology tests.
- After St Marks correspondence, women will be offered a mode of delivery recommendation discussion regarding best options for next delivery and birth plan with a member of PPHS team.
- There are no systematic reviews or randomised controlled trials to suggest the best method of delivery following OASIS. The risk of sustaining a further sphincter injury after a subsequent delivery is 5–7%. Decision-making about the mode of delivery following OASIS should be individualised in consultation with members of the perineal team and the future obstetrician responsible for their obstetric care.
- The role of prophylactic episiotomy in subsequent pregnancies is not known and therefore an
  episiotomy should only be performed if clinically indicated. However, there is evidence that mediolateral episiotomy can reduce the risk of a repeat sphincter injury by 80% (D'Souza 2020) and therefore an episiotomy should be discussed with the woman prior

to delivery. This is particularly important if there are predisposing risk factors for OASI such as big baby, occipitoposterior position, instrumental delivery, shoulder dystocia, fibrotic band or inelastic perineum.

3.10 Risk Management including monitoring women with problems relating to perineal repair

Document an accurate detailed account in the maternity notes using the proforma, which includes:

- Extent of the tear following systematic assessment
- Consent
- Method of repair including analgesia and its effectiveness
- Materials used, swab and needle counts pre and post procedure
- Findings of vaginal and rectal examinations pre and post procedure
- Postoperative care
- A Datix incident form must be completed for **ALL** 3rd/4th degree tears

A Datix incident form must also be completed for any woman returning to hospital with problems relating to all types of perineal repair. These forms are used to monitor rates and causes of returns. These are reviewed by the Risk Midwife and discussed in the Incident Review Meetings. An action plan is formulated and followed up by the Risk Midwife, including addressing training issues if appropriate.

3.11 Documentation of information given regarding support following the repair

The person undertaking the suturing must inform the woman about the extent of the tear, suture material used, personal hygiene, pain relief, pelvic floor exercises and dietary advice. This must be documented on the proforma.

Follow up for women with 1st or 2nd degree tear or episiotomy is provided by the midwife or GP. Women with 3rd and 4th degree tears are seen by the consultant team and by the physiotherapist at 6-11 weeks. The patient information leaflet is part of the postnatal discharge checklist.

3.12 Returns /readmissions with problems following perineal repair

- When a woman returns to triage or is readmitted with problems following perineal repair the Midwife responsible for the woman's care should refer the woman to the Perineal Specialist Midwife (giulia.ciccolini@nhs.net) or the PPHS team (thh.perinatalpelvichealthservice@nhs.net) to review the woman or for advice and to inform them that the woman has returned with issues. If the PPHS staff are not available during the admission, please request a senior doctor review and email to PPHS team so they can follow it up.
- The woman ideally is to be reviewed by the perineal team and a management plan will be documented. A follow up appointment is made at the PPHS Clinic if required, which the midwife responsible for care will arrange using a referral form (appendix 5).
- If there has been wound dehiscence, after review by the PPHS team, the patient will be offered resuturing of the perineum or allowing the wound to heal by secondary

intention depending on the extent of the dehiscence. Resuturing of the wound will be ideally carried out by the PPHS team.

- The Perineal Specialist Midwife monitors the incident rates and reasons for return of women with problems relating to all types of perineal repair.
- A Datix must be completed in line with the Maternity Services Risk Management Strategy for women readmitted to hospital under subcategory “obstetric wound breakdown”.

4. Associated Guidelines, Polices and Documents

- Maternity Specific Mandatory Training Policy Including Skills and Drills
- Aseptic Technique Policy
- Hand Washing Policy
- Bladder Care during Labour and the Postnatal Period
- Antibiotic Guidelines for Obstetrics and Gynaecology
- Procedure for counting swabs, needles & instruments in Maternity – excluding theatres guideline

5. Training Need required

Training for this guideline is provided as described in the Training Needs Analysis.

Staff will be made aware of guidelines during the induction process by their clinical line manager. All staff have a responsibility to check the intranet for the latest version of guidelines when and as required by clinical practice.

6. Implementation and Monitoring

All guidelines, protocols and procedures are available on the intranet. All staff have a responsibility to check the Trust’s intranet and ensure that they are referring to the most recent issue of the document. The weekly general information bulletin includes notification of publication of new/updated documents.

7. References

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8. Appendices.

- Appendix 1 – Repair of Perineal Trauma – 1st & 2nd Degree Tears / Episiotomy proforma
- Appendix 2 – Repair of Perineal Trauma –3rd & 4th Degree Tears proforma
- Appendix 3 – Patient Information
- Appendix 4 – leaflet “care for your perineum: after the birth of your baby”
- Appendix 5 – referral form to PPHS clinic
- Appendix 6 – Sticker for documentation on perineum inspection
- Appendix 7 – Treatment options discussed and offered in the PPHS clinic

The Hillingdon Hospitals
NHS Foundation Trust

APPENDIX 1 REPAIR OF PERINEAL TRAUMA – 1st & 2nd Degree Tears / Episiotomy

PATIENT DETAILS/ADDRESSOGRAPH LABEL STAFF

Patient name		Name	
Hospital number			
Date of birth		Designation	

DECISION FOR REPAIR

Date decided ……. Time decided …….

Verbal consent yes no (please circle)

ANAESTHETIC Spinal Epidural Spinal/epidural Pudendal Local anaesthesia (Volume in mls)

Tested and effective yes no

REPAIR NOTES

Date of Repair …….. **Time started** ……hr

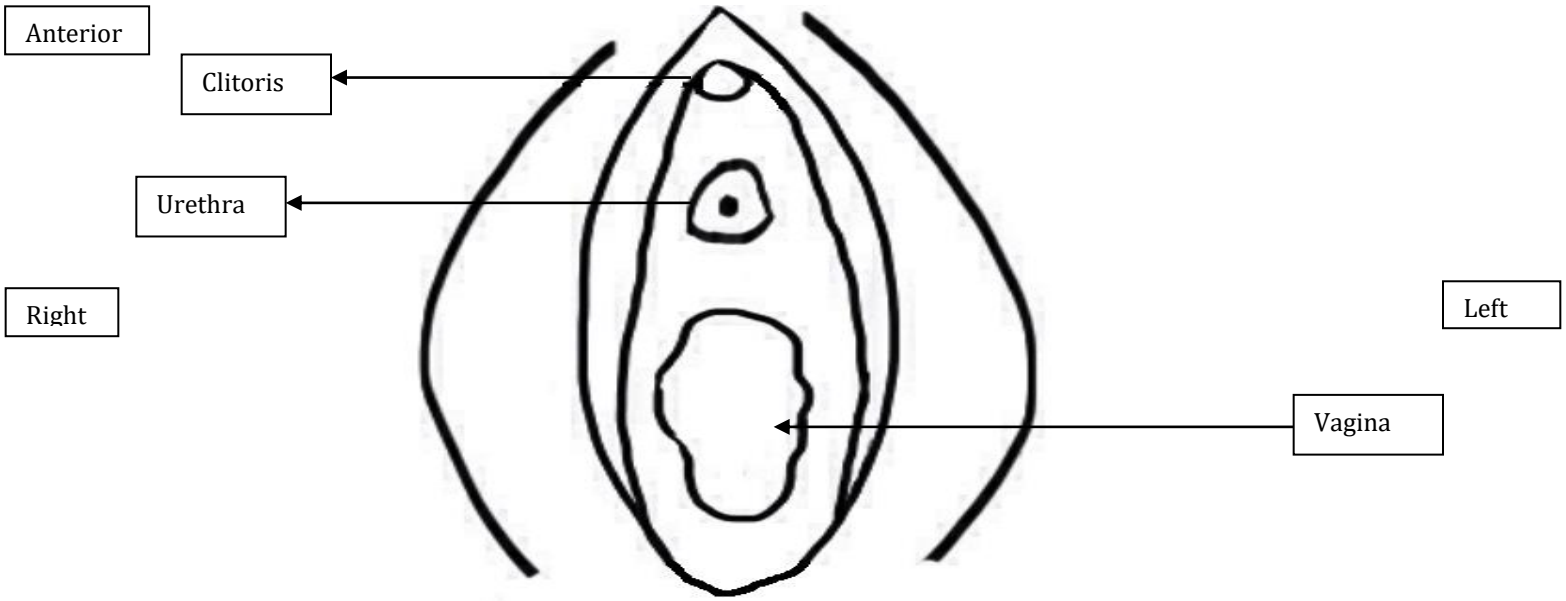
Time completed……..……hr

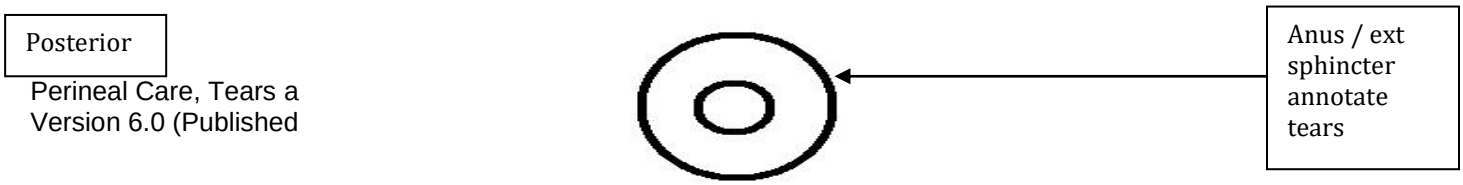
Reason for delay (standard 2hrs) …….

Place of Repair ……. **Classification 1st** degree 2nd degree Episiotomy (Please circle)

PV pre procedure Yes No comment……

PR pre procedure Yes No comment……





BLADDER

Indwelling catheter Yes ? No ? leave for …… hrs

EPISIOTOMY (right mediolateral)

Consent for episiotomy Yes No

Reason for episiotomy ……..

……...…… ? ? ……..

PERINEAL REPAIR

Suture material for vaginal repair 2.0 Vicryl rapide other, please document

Describe method of repair:

Please complete below as appropriate:

Labia……

Vagina……

Perineal muscle……

Perineal skin

…… ? ? ……

POST PROCEDURE

PV post procedure ? Yes ? No

comment……

PR post procedure Yes ? ? No

comment……

Swabs correct pre procedure ? Yes ? No **Signature ……**

No of swabs pre procedure _____ **Signature 2……**

Swabs correct post procedure ? Yes ? No **Signature 1……**

Signature 2……

Needles correct ? Yes ? No

Number of needles used _____ ? ?

Vaginal pack inserted Yes No leave for _____hours

Analgesia given Yes No please state……

Estimated blood loss ? …… ml

Information, including perineal care given to women

SIGNATURE	PRINT NAME	GRADE	DATE

APPENDIX 2 REPAIR OF PERINEAL TRAUMA –THIRD AND FOURTH DEGREE TEARS

PATIENT DETAILS/ADDRESSOGRAPH LABEL STAFF

Patient name		Surgeon 1	
Hospital number		Surgeon 2	
Date of birth		Scrub midwife	
		Runner midwife/HCA	
		Anaesthetist 1	
		Anaesthetist 2	
Midwife 1 at delivery		ODA	
Midwife 2 at delivery		Consultant	

DECISION FOR REPAIR

Date decided ……. Time decided ……. Consultant informed ……..

Grade of operator : ST 3 4 5 6 7 Consultant Written consent yes no

ANAESTHETIC GA Spinal Epidural Spinal/epidural Additional local anaesthetic

Tested and effective yes no

OPERATION NOTES

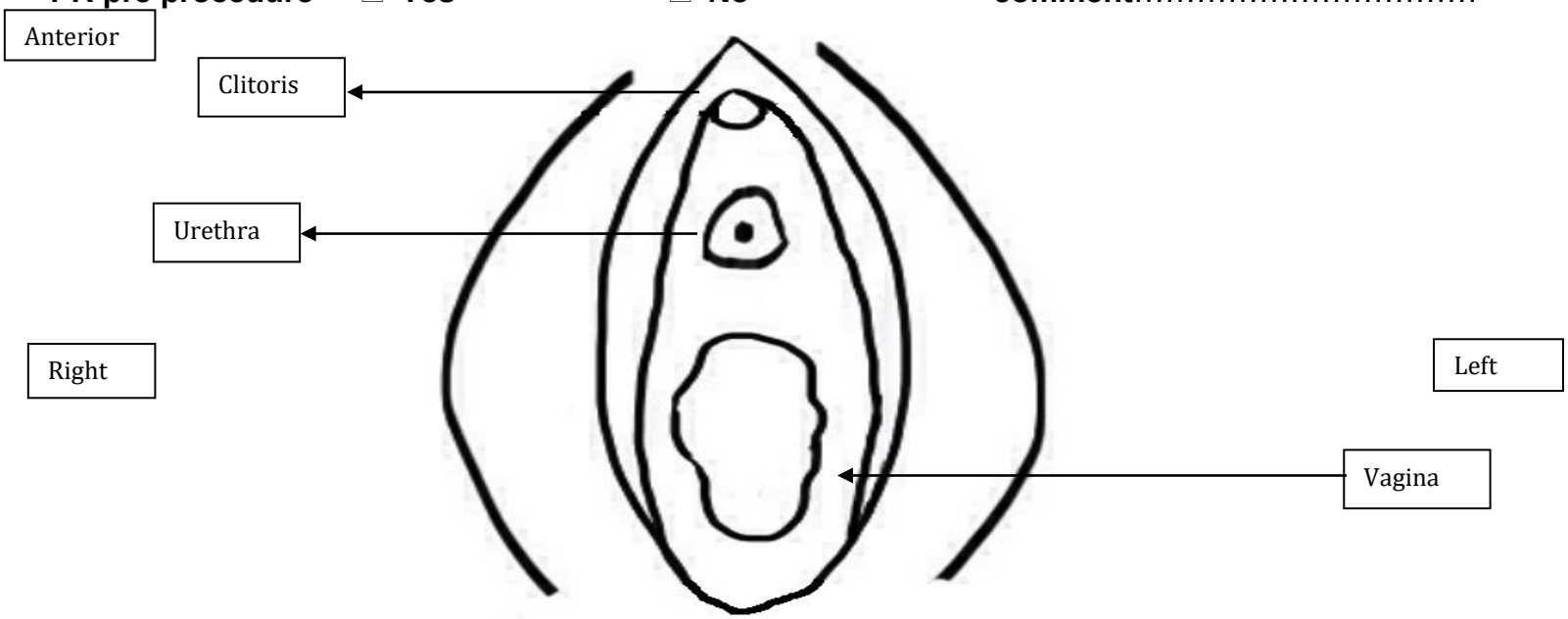
Date of Repair …….. **Time started** ……hr

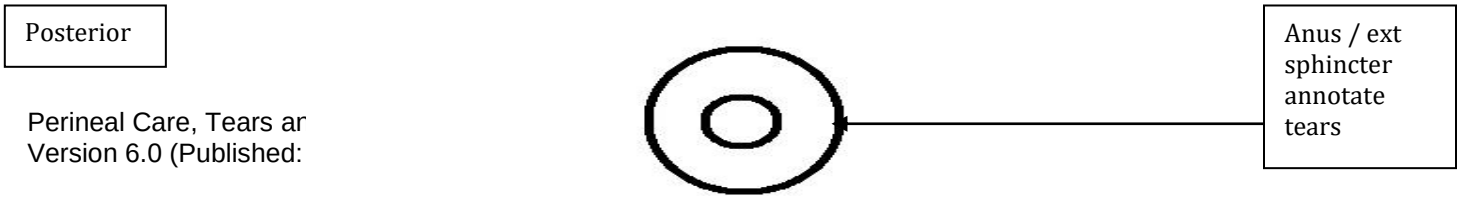
Reason for delay (standard 2hrs) ……

Place of Repair …… **Classification** ……..

PV pre procedure ☐ **Yes** ☐ **No** **comment**……

PR pre procedure ☐ **Yes** ☐ **No** **comment**……





BLADDER

Indwelling catheter ☐ Yes ☐ No leave for …… hrs

EPISIOTOMY (right mediolateral) ☐ Yes ☐ No

PERINEAL TEAR

3rd Degree Tear

- **A** ☐ Less than 50% of external anal sphincter (EAS) torn
- **B** ☐ More than 50% EAS
- **C** ☐ Involves the internal anal sphincter (IAS)

4th Degree Tear

Rectal buttonhole tear ☐ Isolated (anal sphincter intact)
☐ In addition to a third-degree tear

Size of tear (length x width)……cm x…….cm

Suture material for vaginal repair ☐ 2.0 Vicryl rapide ☐ other, please document

Describe……

……

Suture material for sphincter repair ☐ 3.0 PDS ☐ 2.0 Vicryl

Method of EAS repair ☐ overlap ☐ End to end

Method of IAS repair ☐ continuous ☐ End to end

Interrupted 3.0 vicryl rapide to mucosa ☐ Yes ☐ No

Episiotomy/tear closed in layers ☐ Yes ☐ No

PV post procedure ☐ Yes ☐ No comment……

PR post procedure ☐ Yes ☐ No comment……

PR analgesia given ☐ Yes ☐ No comment……

Swabs correct pre procedure ☐ Yes ☐ No **Signature 1……**

No of swabs pre procedure _____ **Signature 2……**

Swabs correct post procedure ☐ Yes ☐ No **Signature 1……**

Signature 2……

Needles correct ☐ Yes ☐ No

Number of needles used Pre- procedure _____

Vaginal pack inserted ☐ Yes ☐ No leave for_____hours

Estimated blood loss ……ml

POST OPERATIVE INSTRUCTIONS

Co-Amoxiclav 7 days

Paracetamol 1g oral qds 14 days

diclofenac MR 75mg bd

Lactulose 10ml bd 14days

NO SUPPOSITORIES OR ENEMAS ?			
Clinical incident form completed ?			
Information, including perineal care given to women			
Postnatal appt 6 weeks (includes assessment for physiotherapy referral)			
SIGNATURE OF LEAD SURGEON	**PRINT NAME**	**GRADE**	**DATE**

Appendix 3 Patient information from the Royal College of Obstetrics and Gynaecology

A third- or fourth-degree tear during birth (also known as obstetric anal sphincter injury – OASI)

About this information

This information is for you if you want to know more about third- or fourth-degree perineal tears (also known as obstetric anal sphincter injury – OASI). It may be helpful if you are a relative or friend of someone who is in this situation.

What is a perineal tear?

Many women experience tears to some extent during childbirth as the baby stretches the vagina. Most tears occur in the perineum, the area between the vaginal opening and the anus (back passage).

Small, skin-deep tears are known as first-degree tears and usually heal naturally. Tears that are deeper and affect the muscle of the perineum are known as second-degree tears. These usually require stitches.

An episiotomy is a cut made by a doctor or midwife through the vaginal wall and perineum to make more space to deliver the baby.

What is a third- or fourth-degree tear?

For some women the tear may be deeper. A tear that also involves the muscle that controls the anus (the anal sphincter) is known as a third-degree tear. If the tear extends further into the lining of the anus or rectum it is known as a fourth-degree tear.

How common are third- or fourth-degree tears?

Overall, a third- or fourth-degree tear occurs in about 3 in 100 women having a vaginal birth. It is slightly more common with a first vaginal birth, occurring in 6 in 100 women, compared with 2 in 100 women who have had a vaginal birth previously.

What increases my risk of a third- or fourth-degree tear?

These types of tears usually occur unexpectedly during birth and most of the time it is not possible to predict when it will happen. However, it is more likely if:

- this is your first vaginal birth
- you are of South Asian origin
- your second stage of labour (the time from when the cervix is fully dilated to birth) is longer than expected.
- you require forceps or a ventouse to help the delivery of your baby – see RCOG patient information *Assisted vaginal birth (ventouse or forceps)* (www.rcog.org.uk/en/patients/patient-leaflets/assisted-vaginal-birth-ventouse-or-forceps)
- one of the baby's shoulders becomes stuck behind your pubic bone, delaying the birth of the baby's body, which is known as shoulder dystocia – see RCOG patient information *Shoulder dystocia* (www.rcog.org.uk/en/patients/patient-leaflets/shoulder-dystocia)
- you have a large baby (over 4 kg or 8 pounds and 13 ounces)
- you have had a third- or fourth-degree tear before.

Could anything have been done to prevent this type of tear?

In most instances, a third- or fourth-degree tear cannot be prevented because it cannot be predicted. However, applying a warm compress to the perineum while you are pushing does appear to reduce the chance of a third- or fourth-degree tear. Your midwife or obstetrician may protect the perineum as your baby's head is delivering and this may also help prevent a tear.

It is unclear whether an episiotomy will prevent a third- or fourth-degree tear from occurring during a normal vaginal birth. An episiotomy will only be performed if necessary, and with your consent.

If you have an assisted birth (ventouse or forceps), you are more likely to have an episiotomy as it may reduce the chance of a third- or fourth-degree tear occurring.

What will happen if I have a third- or fourth-degree tear?

If a third- or fourth-degree tear is suspected or confirmed, this will usually be repaired in the operating theatre. Your doctor will talk to you about this and you will be asked to sign a consent form. You will need an epidural or a spinal anaesthetic, although occasionally a general anaesthetic may be necessary.

You may need a drip in your arm to give you fluids until you feel able to eat and drink. You are likely to need a catheter (tube) in your bladder to drain your urine. This is usually kept in until you are able to walk to the toilet.

After the operation you will be:

- offered pain-relieving drugs such as paracetamol, ibuprofen or diclofenac to relieve any pain
- advised to take a course of antibiotics to reduce the risk of infection because the stitches are very close to the anus
- advised to take laxatives to make it easier and more comfortable to open your bowels.

Once you have opened your bowels and your stitches have been checked to see that they are healing properly, you should be able to go home.

Will I be able to breastfeed?

Yes. None of the treatments offered will prevent you from breastfeeding.

What can I expect afterwards?

After having any tear or an episiotomy, it is normal to feel pain or soreness around the tear or cut for two to three weeks after giving birth, particularly when walking or sitting. Passing urine can also cause stinging. Continue to take your painkillers when you go home.

Most of the stitches are dissolvable and the tear or cut should heal within a few weeks, although this can take longer. The stitches can irritate as healing takes place but this is normal. You may notice some stitch material fall out, which is also normal.

To start with, some women feel that they pass wind more easily or need to rush to the toilet to open their bowels. Most women make a good recovery, particularly if the tear is recognised and repaired at the time: 6–8 in 10 women will have no symptoms a year after birth.

What can help me recover?

Keep the area clean. Have a bath or showers at least once a day and change your sanitary pads regularly (wash your hands both before and after you do so). This will reduce the risk of infection.

You should drink at least 2–3 litres of water every day and eat a healthy balanced diet (fruit, vegetables, cereals, wholemeal bread and pasta). This will ensure that your bowels open regularly and will prevent you from becoming constipated.

Strengthening the muscles around the vagina and anus by doing pelvic floor exercises can help healing. It is important to do pelvic floor exercises as soon as you can after birth. You should be offered physiotherapy advice about pelvic floor exercises to do after surgery.

Looking after a newborn baby and recovering from an operation for a perineal tear can be hard. Support from family and friends can help.

When should I seek medical advice after I go home?

You should contact your midwife or general practitioner if:

- your stitches become more painful or smelly – this may be a sign of an infection
- you cannot control your bowels or flatus (passing wind).

Talk to your GP if you have any other worries or concerns. You can be referred back to the hospital before your follow-up appointment if you wish.

When can I have sex?

In the weeks after having a vaginal birth, many women feel sore, whether they've had a tear or not. If you have had a tear, sex can be uncomfortable for longer. You should wait to have sex until the bleeding has stopped and the tear has healed. This may take several weeks. After that you can have sex when you feel ready to do so.

A small number of women have difficulty having sex and continue to find it painful. Talk to your doctor if this is the case so that you can get the help and support you need.
It is possible to conceive a few weeks after your baby is born, even before you have a period. You may wish to talk with your GP or midwife about contraception or visit your local family planning clinic to discuss this.

Your follow-up appointment

You may be offered a follow-up appointment at the hospital 6–11 weeks after you have had your baby to check that your stitches have healed properly. You will be asked questions about whether you have any problems controlling your bowels. You may be referred to a specialist if you do.

You will also have the opportunity to discuss the birth and any concerns that you may have.

Can I have a vaginal birth in the future?

Most women go on to have a straightforward birth after a third- or fourth-degree tear.

However, there is an increased risk of this happening again in a future pregnancy. Between 5 and 7 in 100 women who have had a third- or fourth-degree tear will have a similar tear in a future pregnancy.

You may wish to consider a vaginal delivery if you have recovered well and do not have any symptoms. If you continue to experience symptoms from the third- or fourth-degree tear, you may wish to consider a planned caesarean section.

You will be able to discuss your options for future births at your follow-up appointment or early in your next pregnancy. Your individual circumstances and preferences will be taken into account.

Appendix 4

MATERNITY SERVICES

CARE OF YOUR PERINEUM: AFTER THE BIRTH OF YOUR BABY

Information for patients, relatives and carers

Introduction

Congratulations on the birth of your baby from the midwives and doctors at Hillingdon Hospital. This leaflet provides you with some advice about how to take care of your perineum after the birth of your baby. Please ask our team if you have any questions about the information below. If you need more help, please use the contact details at the end of this leaflet.

Where is your perineum?

The perineum is the part of your body between the opening to your vagina and your anus (or back passage). The perineum can tear when you are giving birth, or sometimes your healthcare professional might recommend a cut to your perineum (episiotomy) to make more room for your baby to be born. After the birth of your baby a healthcare professional will offer you an examination to see if you have a tear. They will advise you if you need stitches. Most tears heal within 6 weeks with no long-term problems.

Some potential common symptoms experienced are listed below. If you do experience any of these please let your healthcare provider know and they can refer you for further help:

- pain / soreness around the vagina, back passage and tailbone
- a feeling that you need to rush to the toilet to open your bladder or bowels urgently
- difficulty controlling wind
- difficulty opening your bowels and / or needing to strain
- discomfort with sexual intercourse

How to manage your pain while in hospital and at home

1. Use the painkillers prescribed.
2. Avoid sitting or standing for long periods of time for the first two weeks. To help with sitting, place towels or pillows under the length of each thigh, to raise you up and take the pressure off of the area. We do not recommend sitting on a rubber ring.
3. Using ice packs for the first few days can help with pain and improve the healing of the wound by reducing swelling. To use ice over the perineum, wrap it in a clean, damp towel and place it onto the area or dampen some sanitary towels, put them in the freezer then use these as an ice pack. You must cover the sanitary towel in a face cloth or a few rolls of paper towel before using it to prevent ice burns.
4. Place the ice pack over the perineum for 10 minutes, and repeat every 2-3 hours as indicated. 10 minutes is the time needed to reduce the swelling, after this the area will just get cold.

Wound care

If you had a tear or episiotomy, you may have had stitches to help promote healing, although some minor tears heal best without stiches. The perineum is usually repaired using a “continuous stitch” – this means that one length of stitch material is used to repair vagina, perineal muscles and skin. This should

result in less pain during the first few days compared to have individual stitches. However, in some cases, individual stitches may be necessary to ensure good repair.

The material used to stitch your perineum is absorbable, so usually your stitches don't need to be removed. Occasionally the stitches may need removal if they are causing a problem, such as pain in the area of the stitch or if there are concerns about infection.

Your stitches should feel more comfortable within 10-14 days of having your baby, and the suture material should have dissolved completely within the first 6 weeks after birth. However, up to 20% of women may experience pain for up to 8 weeks.

Stitches in the sphincter muscle take longer to dissolve (around three months). Sometimes these can be felt through the skin near the back passage, when wiping your bottom - this is normal.

Washing

- It is normal to have bleeding and vaginal discharge. You should keep the area clean and dry by washing once a day and changing your sanitary pad regularly
- You should use warm water to wash the area - do not use salt water
- Do not soak in a bath for up to 14 days. A shower head, or bucket can be used to apply water to the perineum.
- When drying the area, simply pat dry with a clean towel
- After opening your bowels, wash yourself from front to the back, to avoid stool (poo) getting into the perineal wound area or near the urinary passage. This will reduce the risk of wound or urinary tract infection.
- Always wash your hands after using the bathroom or toilet facilities to avoid spreading bacteria to the wound.

What if you have any concerns about your perineum

Occasionally, the perineum may become infected or the wound may open. This is usually treated first with antibiotics and the wound is allowed to heal naturally. There are some occasions when it is clinically advised to restitch the wound: in this case, it will be a shared decision between you and the senior doctor with a discussion on benefits and complications.

Some women feel there are anatomical changes to the vaginal and perineal area which cause them concern, or perineal pain which is not resolving with pain relief. Others may also experience pain when they resume sexual activity.

If you are having problems with your stitches or experiencing any concerns, please discuss this with your community midwife or GP- women usually know their own bodies.

You can contact Hillingdon maternity triage if you are experiencing any problems with your stitches. You can also speak to your midwife or your GP to book an appointment for you to attend the Perinatal Pelvic Health Service (PPHS) clinic- also see email at the end of this leaflet.

Scar massage

Scar massage may help to reduce discomfort after having a baby, especially when you start to resume sexual intercourse. However, be sure that your stitches have healed completely before you start. The aim is start to bring blood flow to this area , which provides oxygen to help wound healing.

You want to gently start to break down the scar with pressure and dissipate or 'spread out' the scar tissue that has formed.

How to prepare?

First of all, a mirror can help you to locate the area between the vagina and the anus so you know what you are doing. Imagine your perineum represents a clock face.

Hands should be clean with nails clipped. Make sure your bladder is empty before starting. Some women find having a warm bath beforehand can help relax and soften the tissue.

It is recommended that you use a carrier massage oils to lubricate: rose-hip oil, olive oil, vitamin E oil, coconut or almond oil and shea butter are all good to be used (Be careful with baby's allergy if you use Coconut or almond oil as well as Shea Butter, they contain nuts).

Which position?

Position yourself so that you are comfortable. You could try:

- lying down flat
- Squatting down (supporting yourself by leaning back against a wall or forwards on a bed or chair)
- Sitting on the toilet
- Standing with one foot raised on a chair / toilet
- Leaning back or kneeling on all fours

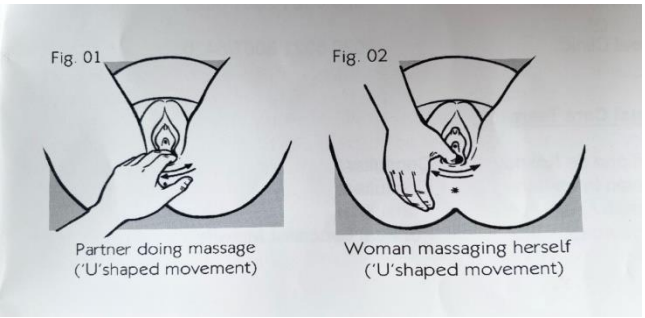
How to do it?

You need to gently apply moderate pressure (no more than 3/10 in intensity) and massage along the scar, in an up-down direction on your perineum externally.

Where you feel more pressure, it's were you will need to work most. If you find difficult to touch your scar, start with "labial pull", so gently pull the labia up and down and make circle to make movements and become comfortable with touching.

Place your thumb at the vaginal entrance and make a "U" shape to gently stretch the muscles and scar. Do the massage with clean hands, in a relaxed position, every day for at least 2 minutes scar massage.

You can do the massage daily or at least until you are comfortable with the scar.



Taking care of your bladder

After delivery your midwife will give you a bowl to measure your urine in. It is important for us to know the volume of urine to ensure your bladder is working properly.

You may have a catheter post-delivery. After the catheter is removed it is important you pass urine within six hours. If you don't you must tell your midwife or doctor immediately.

If you are having difficulty passing urine or have not had an urge to pass urine **four hours after** your catheter has been removed try:

- going to sit on the toilet, relaxing and leaning forwards
- turning on the taps so you can hear running water or pulling lightly on the pubic hair (both of these can help to stimulate an urge)
- rocking forwards and backwards on the toilet
- gently tapping over the bladder near your pubic bone for a few minutes

If you have pain when you pass urine, pour lukewarm water over the perineum as you pass urine or just after. Please tell your midwife about any pain.

It is important to keep this area clean and dry in between toilet visits and change sanitary pads regularly.

It is important to drink one-and-a-half to two litres of water in the first 24 hours - this should be spaced throughout the day.

After the first 24 hours increase your fluids to two to three litres per day if you are breastfeeding.

How to manage your bowels

Many ladies are anxious about opening their bowels for the first time after delivery. It is important that you try not to put off opening your bowels once you have had the urge to go, as this can make you more constipated.

Use the laxatives prescribed as you need to ensure that your stools remain soft but not watery. It is important to keep your stools like a 'toothpaste' consistency.

To help to prevent constipation you will need to maintain a good fluid intake and healthy diet rich in fibre.

Make sure you sit in a good position on the toile to ensure you completely empty your bowels.

The ideal position is:

- knees higher than your hips (to do this place your feet on a step or point your toes)
- lean forward and put your elbows on your knees
- bulge your abdomen outwards and straighten your spine
- if you have discomfort, hold a sanitary pad or a wad of tissues with your hand and apply pressure to the vagina and perineum

Sexual intercourse

We recommend you can return to sex when feeling comfortable and ready, which for most is after 6 weeks when the stiches have fully healed, but can be earlier for some. There is no need to delay intercourse beyond 12 weeks unless you have been advised to at your check-up.

Some couples may worry about having sex for the first time and you may find it slightly uncomfortable for the first few times. It can be helpful to use an additional water-based lubricant and start gently in order to avoid any problems.

Exercises

Pelvic floor muscles

These muscles help to control the bladder and provide support to the uterus, bladder and bowel.

After a vaginal birth you should start pelvic floor exercises as soon as the catheter has been removed and you have passed urine. The exercises will help reduce swelling and pain and prevent / treat incontinence.

You should aim to do all of the exercises three times a day.

Imagine you are trying to stop yourself passing wind and then pull forward as though you are also trying to stop your flow of urine. This closes and draws up the back passage and vagina. Immediately after birth, start with some very gentle squeeze and relax 8 times in a row, three times a day for the first two weeks. Then after this start to build up the strength by doing the exercises below…

You need to exercise this muscle in two ways:

1. Slow holds: Hold the muscle in for as long as you can up to 10 seconds. Release slowly and rest for at least five seconds between each one. Aim to do 10 in a row.

2. Fast squeezes: Squeeze the same muscles but strong and quickly, releasing straight away. Aim to do 10 in a row.

You can do these exercises almost anywhere and anytime but not when passing urine. In the beginning you will find these exercises easier to do when lying down or sitting.

Contacts

Labour ward: 01895 279333

Triage: 01895 279054

Community midwives office: 01895 279472

Perineal clinic : thh.perinatalpelvichealthservice@nhs.net

Perineal care team

Miss Isra Ali – Consultant

Dr Adanna Okeahialam – Registrar

Samantha Vincent – Pelvic Health Specialist Physiotherapist

Giulia Ciccolini – Pelvic Health Specialist Midwife

Appendix 5

Perinatal Pelvic Health Service (PPHS) Referral

The Hillingdon Hospital, North West London

ALL DETAILS must be completed clearly in the table below in **BLOCK CAPITALS**

Patient Name		**Referral made from PLEASE CIRCLE** Antenatal clinic Labour ward Postnatal ward Community Health Visitor Other:
Hospital Number		
Date of birth		
Address		**EDD & Gestation :** **Days / weeks postnatal :** **(Please refer to GP if over 1 year)**
Daytime contact number		**Consultant**
Reason for Referral PLEASE CIRCLE Antenatal ● Previous 3rd/4th degree tear ● Previous perineal care after complex tear, if still symptomatic ● Vagina feeling heavy/ full / prolapse ● Urine leaking ● Poor control and / or incontinence of wind/stools ● Persistent perineal pain ● Painful sexual intercourse / internal examination ● Vaginismus Postnatal ● Perineal wound breakdown ● perineal wound haematoma ● Perineal wound infection ● 3rd and 4th degree tears ● Postnatal dyspareunia ● Persistent Perineal Pain ● Postnatal faecal / flatus incontinence ● Postnatal urinary incontinence ● Prolapse ● Retention: please specify amount retained and length of IDC Details: _____		How long has the patient had these symptoms ? _____ Does this patient require an interpreter ? Language_____ Please include any other details about referral : _____
Relevant History e.g. Obstetric / Medical / Social		
Referrer		**Date of referral**

Email completed referral forms to **thh.perinatalpelvichealthservice@nhs.net**

Appendix 6

Sticker for documentation on perineum inspection

Perineal tear (PLEASE CIRCLE) : 1st/ 2nd / 3rd (A, B, C) / 4th / episiotomy
Other tears: labial/ paraurethral / urethral/ clitorideal/ cervical
Have you inspected the perineum?

- ☐ Yes, with consent
- ☐ No please state reason _____

If the patient has declined the perineal inspection prior to discharge, please tick here to confirm you communicate the risks associated with not having her perineum inspected.

Condition of perineum:

☐ Healing well	☐ Suture in situ	☐ Not sutured	☐ swollen
☐ painful	☐ infected	☐ bruised	☐ gaping
☐ loose sutures	☐ defect (eg. Misalignment, skin tag, over granulation tissue)	☐ others: _____ __	

If any concerns, escalate to senior doctor/pelvic health specialist midwife/refer to PPHS clinic (PLEASE CIRCLE).

Appendix 7

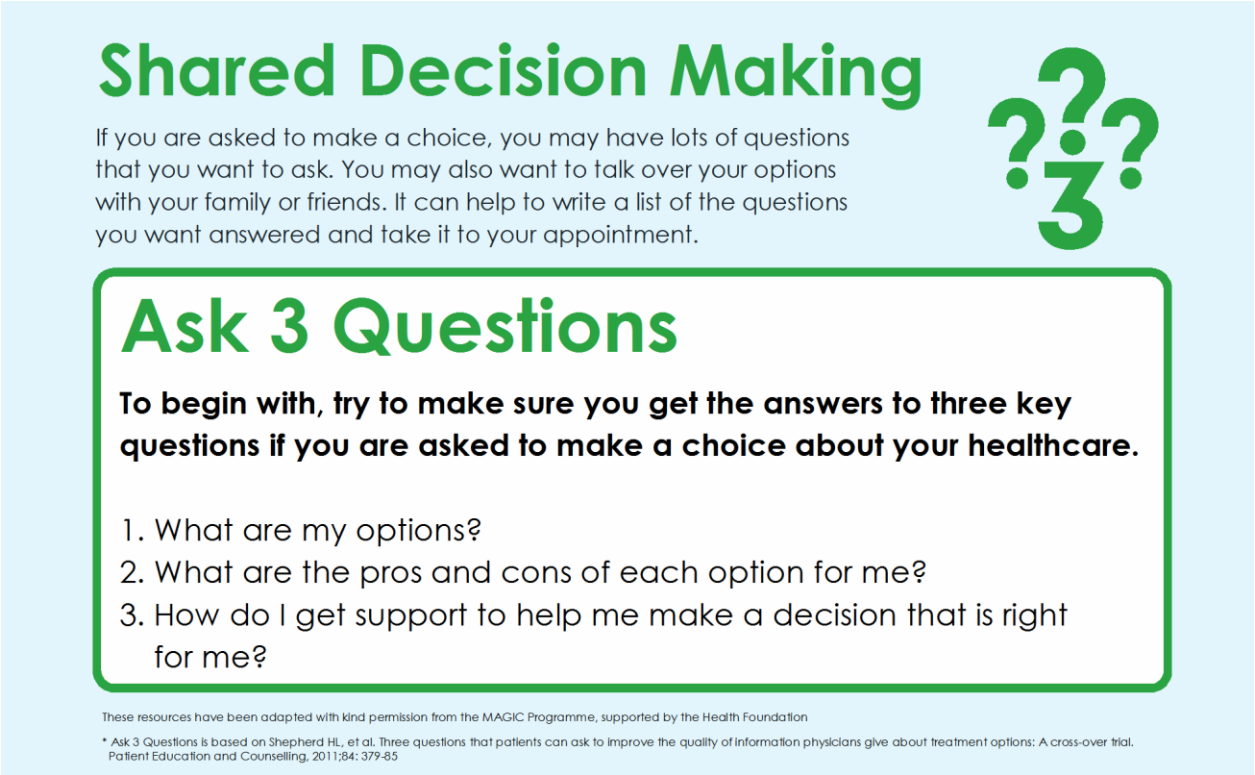
Treatment options discussed and offered in the PPHS clinic

Perineal breakdown	Perineal resuturing vs conservative management Pain management Psychotherapy referral if required
Perineal pain	Targeted perineal/scar massage Silver nitrate for granulation tissues Excision of skin tag/perineal adhesion Correction of misalignment/loose sutures removal Instillagel/lidocaine gel/ointment Vaginal oestrogen Local injections of local anaesthesia, steroids and hyaluronidase Perineal birth plan discussion for next delivery
Sexual concerns	Instillagel Targeted perineal/vaginal massage Psychosexual counselling referral Physiotherapy
Bladder symptoms	Physiotherapy Urodynamics Bladder diary/training Discussion of surgery options
Bowel symptoms	Physiotherapy Dietary/nutrition advice Haemorrhoids treatment Endoanal USS and anorectal physiology
Prolapse symptoms	Physiotherapy Vaginal oestrogen Pessaries Discussion of surgery options
OASI	Endoanal USS and anorectal physiology Mode of delivery conversation for next birth Physiotherapy Consultant follow up at 6-11 weeks

Women who have undergone an obstetric anal sphincter repair are given an appointment to be seen in the PPHS clinic at Cumberledge, run by Miss Isra Ali once a month, 6-11 weeks postnatally. These appointments are made by the administrative staff from labour ward with the date/time given to the woman prior to discharge or sent via post after discharge.
If women have sustained a 4th degree tear or report anorectal symptoms at follow-up, they are referred for an endoanal ultrasound scan and anorectal physiology testing at St Marks.
Where there are other concerns about birth-related perineal trauma e.g. perineal pain, infection or wound dehiscence, women may be referred to the specialist midwife at the PPHS clinic.

Should there be queries or concerns, please contact either Miss Isra Ali (isra.ali1@nhs.net) or Giulia Ciccolini, perineal specialist midwife, on giulia.ciccolini@nhs.net or thh.perinatalpelvichealthservice@nhs.net .

Making a Choice



AQuA Advancing Quality Alliance · Right Care Shared Decision Making Programme · NHS

http://www.advancingqualityalliance.nhs.uk/SDM/

Further Information and support
Bladder and Bowel Foundation: www.bladderandbowelfoundation.org
RCOG patient information:

- Assisted vaginal birth (ventouse or forceps): www.rcog.org.uk/en/patients/patient-leaflets/assisted-vaginal-birth-ventouse-or-forceps
- Shoulder dystocia: www.rcog.org.uk/en/patients/patient-leaflets/shoulder-dystocia

Sources and acknowledgements

This information has been developed by the RCOG Patient Information Committee. It is based on the RCOG Green-top Clinical Guideline *The Management of Third- and Fourth-Degree Perineal Tears* (June 2015). The guideline contains a full list of the sources of evidence we have used. You can find it online at: www.rcog.org.uk/en/guidelines-research-services/guidelines/gtg29.

This leaflet was reviewed before publication by women attending clinics in Ayrshire, London and Staffordshire, and by the RCOG Women's Network.

The RCOG produces guidelines as an educational aid to good clinical practice. They present recognised methods and techniques of clinical practice, based on published evidence, for consideration by obstetricians and gynaecologists and other relevant health professionals. This means that RCOG guidelines are unlike protocols or guidelines issued by employers, as they are not intended to be prescriptive directions defining a single course of management.

A glossary of all medical terms is available on the RCOG website at: www.rcog.org.uk/womens-health/patient-information/medical-terms-explained.